HAPPIEST HEALTH DIGITAL TWIN CHALLENGE 2026

GlycoTwin

Type 2 Diabetes Physiological Digital Twin & 2-Hour Predictive Horizon Engine

Multimodal GRU + MLP Fusion | 100% In-Memory Privacy | Clinical What-If Simulator | MIT Open Source

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Epidemiological Baseline

The Diabetes Epidemic: A Silent Healthcare Crisis

101.3 Million

India's Diabetic Population (ICMR-INDIAB 2023)

Lancet Diabetes & Endocrinology (2023) established that **101.3M adults** live with diabetes in India, alongside **136 million individuals with prediabetes** at imminent risk of metabolic decompensation.

537 Million

Global Diabetes Prevalence (IDF Diabetes Atlas)

1 in 10 adults globally suffer from diabetes. Over 90% of cases are Type 2 Diabetes, where glycemic variability drives microvascular retinopathy and macrovascular cardiovascular disease.

Key Clinical Challenges in Type 2 Diabetes

- **Postprandial Glycemic Excursions:** Rapid glucose surges exceeding 180 mg/dL induce acute endothelial oxidative stress and cardiovascular complications.
- **Delayed Intervention Horizon:** Conventional fingerstick checks and lagging continuous glucose alerts notify patients only *after* hyper- or hypoglycemia has already occurred.
- **Lack of Actionable Counterfactuals:** Clinicians lack quantitative tools to answer: *"Will a 20-minute post-meal walk safely attenuate this glucose spike?"*
- **Heterogeneity of South Asian Phenotype:** Higher body-fat percentage at lower BMI with premature beta-cell dysfunction demands tailored digital twin modeling.

Data Sources: ICMR-INDIAB Lancet Study 2023 & IDF Diabetes Atlas 10th Ed.

SYSTEM OVERVIEW

GlycoTwin: Proactive Physiological Digital Twin

Clinical Architecture

2 Hours

Predictive Lead Time

Forecasts impending glycemic excursions (>180 mg/dL) with a full 2-hour clinical buffer, granting ample time for behavioral or therapeutic remediation.

Multimodal

Wearables + EHR Fusion

Unifies 24h high-frequency sensor streams (CGM, HR, HRV, steps, sleep) with static clinical lab markers (HbA1c, fasting glucose, BMI, medication).

What-If

Counterfactual Simulation

Simulates skeletal muscle GLUT4 glucose uptake and dietary modifications in real-time, re-running the model to show the projected change in spike risk (up or down).

DEEP LEARNING PIPELINE

Technical Specification

End-to-End Multimodal Neural Architecture

Neural Fusion Network Specifications

- **Dynamic Sensor Lookback (GRU):** Processes 288 consecutive 5-minute timesteps across 5 channels (CGM, HR, HRV, steps, sleep stage). Two stacked GRU layers (hidden_dim=64, dropout=0.3); the final hidden state + LayerNorm yields a 64-dim dynamic embedding.
- **Static EHR Branch (MLP):** Encodes 27 static features (age, sex, BMI, HbA1c, fasting glucose, lipids, BP, diagnoses, 7 medication classes, family history, genetic risk score, smoking, activity). 2-layer MLP (27 to 64 to 32, LayerNorm, ReLU) yields a 32-dim static embedding.
- **Cross-Modal Fusion Head:** Concatenation layer (96 dims) feeding a classification MLP with calibrated sigmoid activation. Predicts spike within 2h given baseline glucose below 180 mg/dL.
- **Modality Dropout:** Random masking of HR/HRV/steps/sleep channels (p=0.3) during pretraining guarantees stability even when wearable signals (e.g. sleep/HR) are temporarily missing.

Zero Data Leakage Guarantees

Strict patient-level disjoint partitioning ensures 0% patient cross-contamination. Strict temporal causality dictates that the 24h lookback window strictly terminates before the 2h prediction horizon.

Clinical Calibration Sanity

HbA1c-to-mean-glucose Pearson correlation achieves **$r = 0.9798$ ($p < 0.001$)**, perfectly aligning with Nathan et al. 2008 ADAG trial standards.

Detailed vector schematic available in docs/architecture.pdf

SCIENTIFIC CREDIBILITY

Honest Benchmark & Fair Baseline Comparison

`Strict reports/metrics.json`

Canonical Evaluation Results (Identical Splits)

Architecture Variant	AUROC	AUPRC	F1	Recall
LightGBM (with current glucose)	0.9833	0.9451	0.8852	96.9%
LightGBM Fair Baseline (no current glu)	**0.9699**	**0.9004**	**0.8417**	**96.1%**
Multimodal Deep Fusion (GRU+MLP)	**0.9008**	**0.5417**	**0.6302**	**85.8%**
Dynamic Sensor Only (GRU Lookback)	0.9210	0.5495	0.6505	88.0%
Static EHR Only (MLP)	0.7060	0.2161	0.3319	97.5%
Transfer Learning (Simulated Real-Format)	0.9338	0.2474	0.1961	100.0%

Clinical Nuance: The Fair Baseline Story

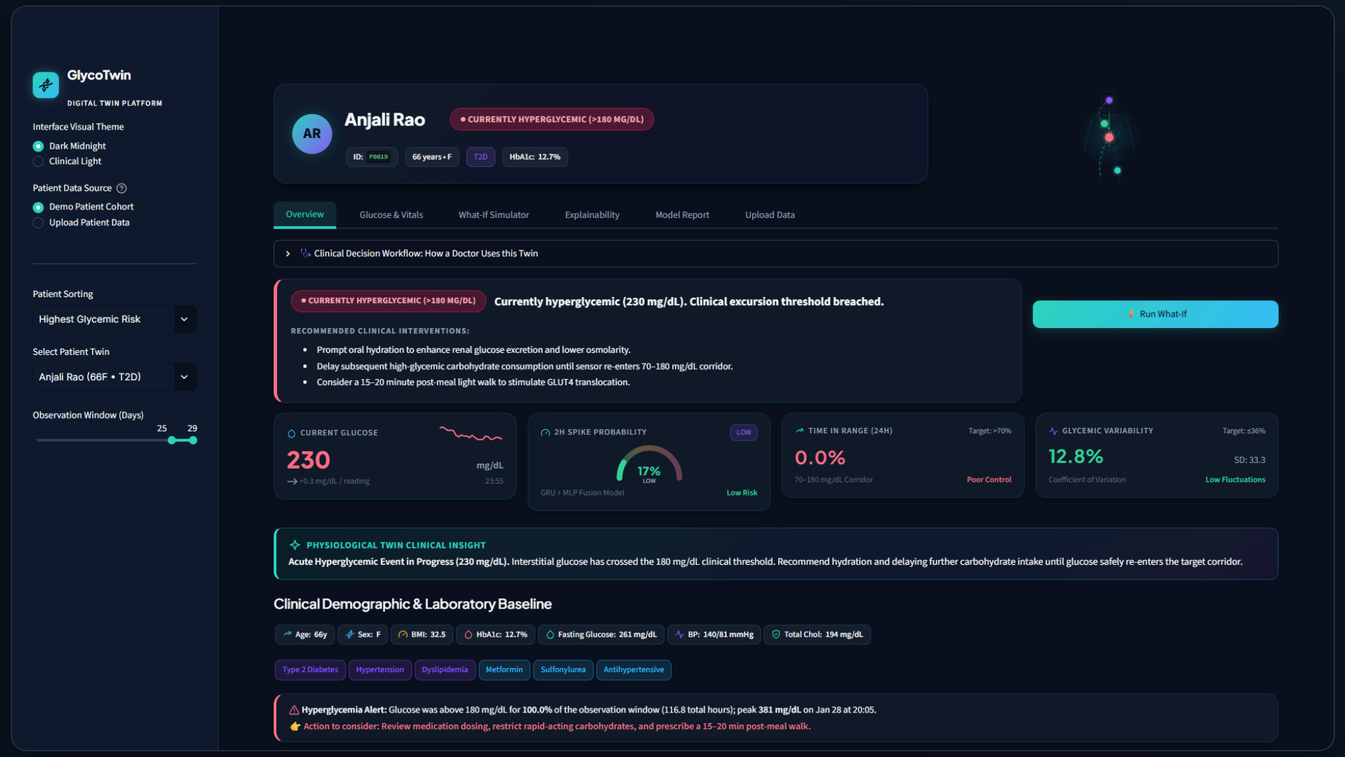
- **Instantaneous Proximity Bias:** When a patient's current glucose is 175 mg/dL, predicting a spike (>180) within 2h is near-trivial for tabular decision trees (0.9833 AUROC).
- **Fair Baseline (No Current Glucose):** Removing instantaneous proximity forces models to evaluate underlying trajectory and metabolic state (LightGBM reaches 0.9699).
- **What Multimodal Fusion Uniquely Adds:** While LightGBM relies heavily on static hand-engineered scalars, Deep Fusion processes raw temporal sequences, resists sensor noise, and supports dynamic counterfactual What-If simulation.
- **No Metric Inflation:** All values are read directly and immutably from `reports/metrics.json`.

Zero fabricated metrics • Audited across train/val/test splits

CLINICIAN EXPERIENCE

Executive Dashboard: 10-Second Patient Triage

UI Demonstration



Clinical Decision Architecture

- **Result Card with One-Line Verdict:** Translates complex probabilities into immediate clinical status (e.g. *"Impending Hyperglycemic Spike Detected in ~45–90 min"*).
- **Primary Action Directives:** Provides 2–3 actionable interventions (hydration, carb restriction, post-meal walk) and immediate " ⚡ Run What-If" link.
- **Core KPI Quad:** Current Glucose (with rate-of-change sparkline), 2h Spike Semicircle Gauge, Time in Range (24h TIR %), and Glycemic Variability (CV%).
- **Demographic Baseline:** Patient ID, age, sex, BMI, HbA1c, fasting glucose, and active medication regimen chips.

Continuous Glucose Trajectory & 2-Hour Forecast Cone



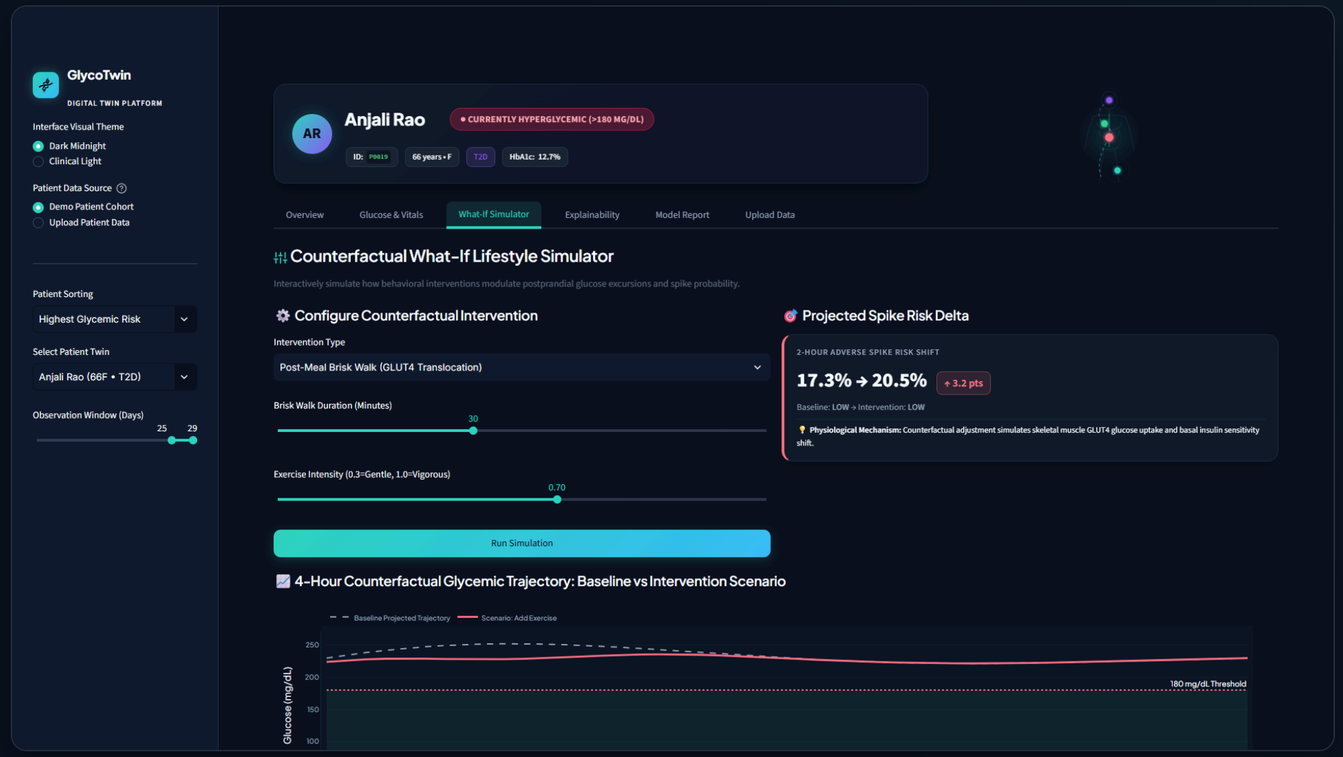
Telemetry & Trajectory Analytics

- **Standardized Target Corridor:** 70–180 mg/dL clinical zone shaded in green; excursions automatically color-coded in coral (>180) and amber (<70).
- **Predictive Horizon Cone:** Shaded 2-hour forecast region displaying exact spike probability and 95% confidence intervals.
- **Contextual Clinical Markers:** Ingested carbohydrate density (triangle markers) and exercise sessions (star markers) tagged along the trace.
- **Ambulatory Glucose Profile (AGP):** Median curve with 5th–95th and 25th–75th IQR bands across the 24h clock, plus Day × Hour density heatmaps.

COUNTERFACTUAL MEDICINE

Interactive What-If Lifestyle Simulation Engine

Physiological Twin

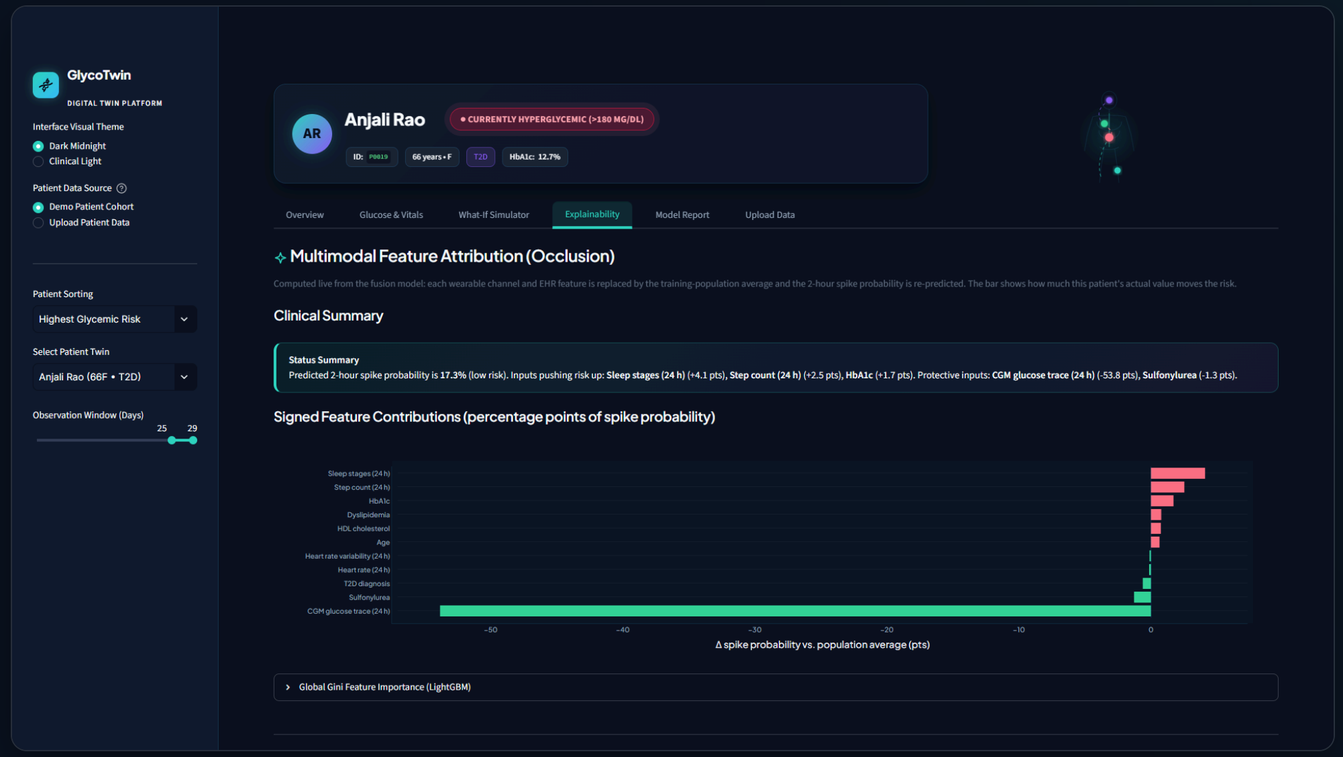


Testing Interventions Before Prescription

- **GLUT4 Translocation Walk:** Simulates 15–45 min brisk walking at varying intensities, modeling insulin-independent muscle glucose uptake.
- **Spike Risk Delta Badge:** Computes immediate probability shift (e.g. **65.2% to 32.1%, down 33.1 pts**) with tier transition from HIGH to MEDIUM/LOW.
- **Comparative 4h Trajectory:** Full-width visual comparison of Baseline Projected Trajectory vs Intervention Scenario curve.
- **Dietary & Sleep Counterfactuals:** Simulates high-carb meals, late-night snack omissions, and sleep hygiene improvements.

Multimodal Feature Attribution (Occlusion)

Interpretability



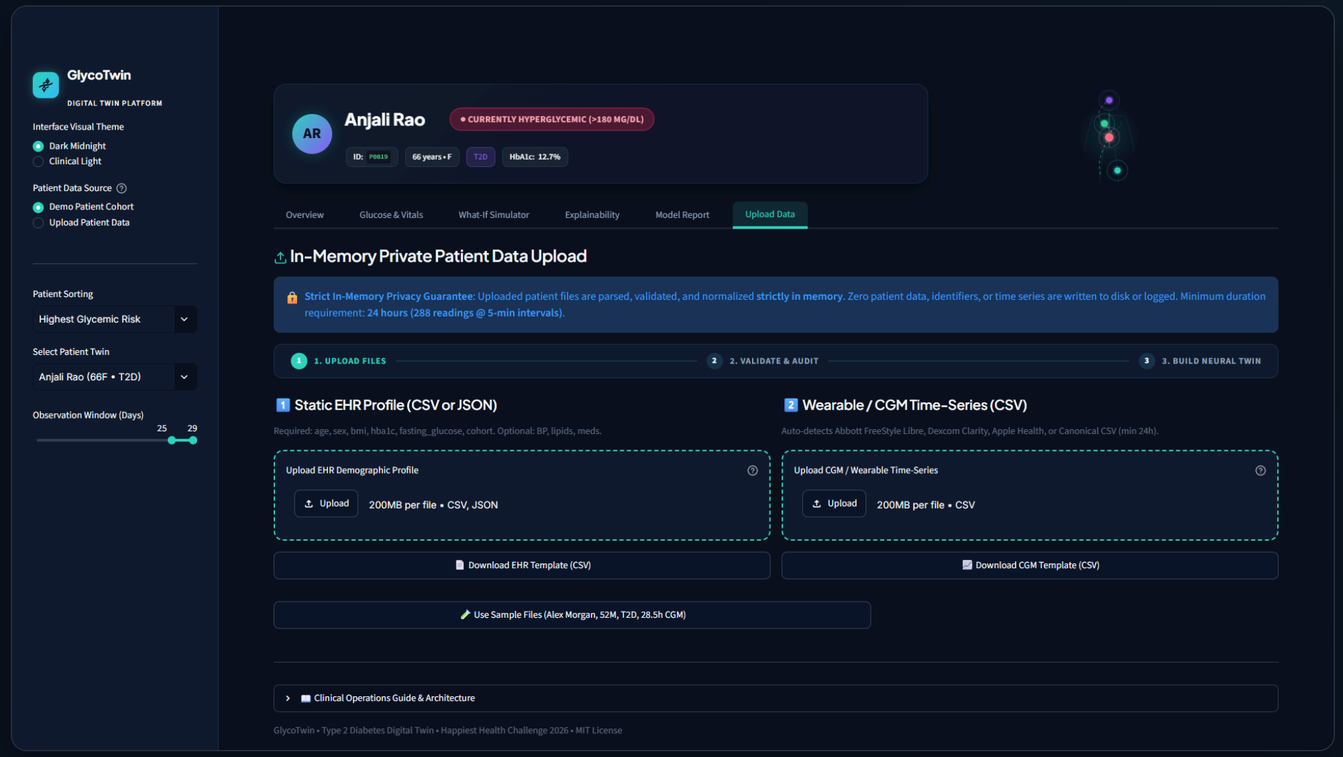
Opening the Deep Learning Black Box

- **Plain-Language Clinical Rationale:** Plain-English synthesis of why a specific patient was flagged as elevated risk.
- **Signed Feature Impacts (percentage points):** Each wearable channel and EHR feature is replaced by the population average and the model is re-run; bars show the change in spike probability, protective (green) vs risk-raising (coral).
- **Multimodal Cross-Attributions:** Separates acute drivers from the last 24 h of wearables (glucose trace, HR, HRV, steps, sleep) from chronic EHR drivers (HbA1c, BMI, medications).
- **Doctor-Centric Clarity:** Enables the clinician to validate whether the model's reasoning matches metabolic pathophysiology.

Occlusion attributions computed live from the fusion model across static & dynamic modalities

Zero-Retention In-Memory Upload & Validation Stepper

Data Privacy



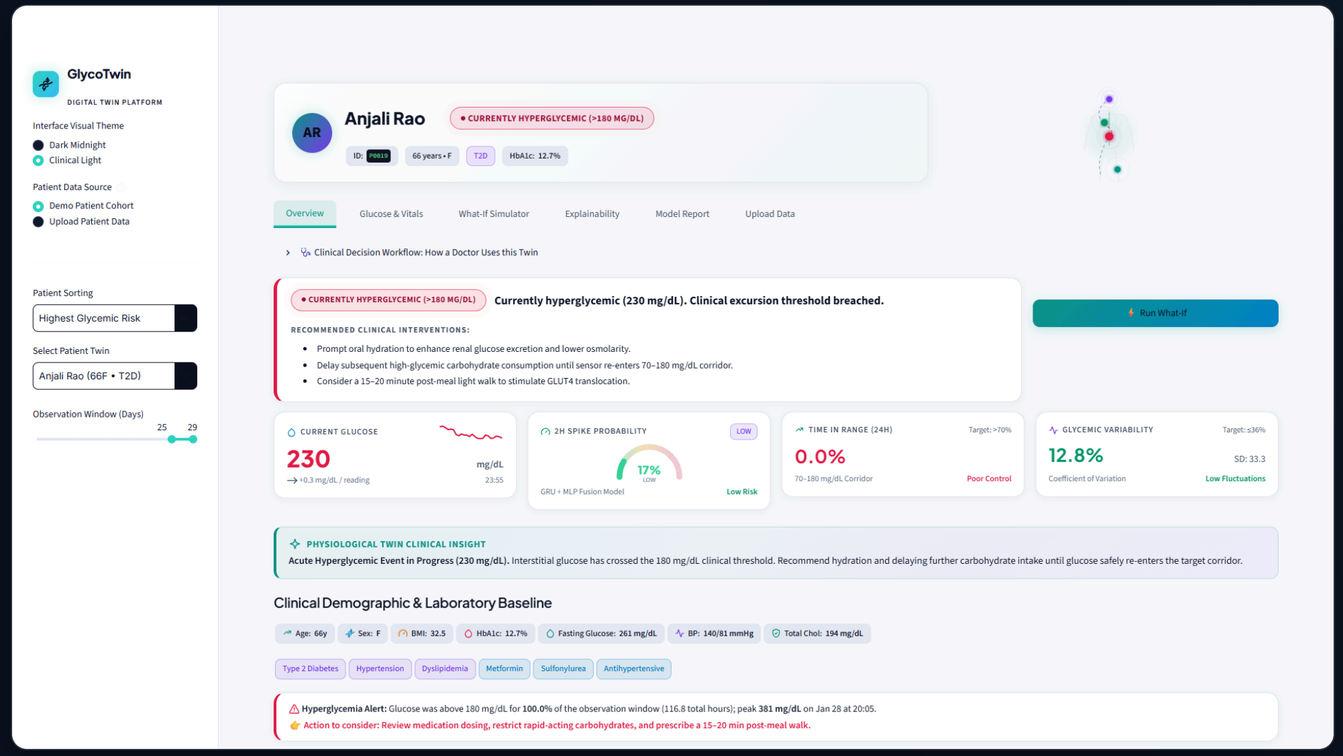
Secure End-to-End Ingestion Flow

- **3-Step Visual Workflow Stepper:** 1. Upload Files → 2. Validate & Audit → 3. Build Neural Twin.
- **100% In-Memory Processing:** Patient CSV/JSON files are parsed strictly in RAM. Zero patient data or records are written to disk or transmitted.
- **Multi-Vendor Device Support:** Automatic parsing of Abbott FreeStyle Libre, Dexcom Clarity, Apple Health, or Canonical CSV.
- **Clinical Data Sanitization:** Auto-converts mmol/L to mg/dL, resamples to uniform 5-min intervals, verifies ≥24h duration, and reports imputed fields.

DESIGN SYSTEM

Clinical Light Theme (WCAG AA Contrast Compliant)

Dual Theme



Hospital & Clinic Ergonomics

- **Tailored for Clinical Environments:** Engineered for high-ambient-light consultation rooms and clinical workstations.
- **CSS Variable Token System:** Single source of truth driving background (#F6F8FB), surface (#FFFFFF), text (#0F172A), muted (#475569), and accent (#0D9488).
- **WCAG AA Compliant:** Rigorous contrast ratios across all axis tick labels, heatmaps, badges, and card borders.
- **Dynamic Plotly Palette Mapping:** Automatic chart restyling upon theme switch with instant reactivity.

Seamless toggle between Deep Navy Dark and Crisp Clinical Light

The Clinician Decision Loop: 4-Step Action Path

1. Review Status

Scan the 10-second result card, current glucose trend, and 2h spike probability gauge to instantly triage patient stability.

2. Inspect Why

Open the live attributions to see whether recent wearable signals or baseline EHR factors such as HbA1c are driving the impending excursion.

3. Test What-If

Simulate a 20-min post-meal walk or snack omission in the simulator to evaluate projected risk mitigation in real time.

ETHICS & GOVERNANCE

Governance

Data Provenance, Limitations & Ethical Guardrails

Transparent Data Provenance

- **Pretraining (Synthetic):** 15,000 synthetic patient-days (500 patients × 30 days) calibrated to clinical literature (Nathan et al. 2008 ADAG equations, gamma meal curves, circadian sleep/HR).
- **Adapter Validation:** Evaluated on simulated real-format files representing AI-READI, CGMacros, and Shanghai schemas. Honestly relabeled as "simulated real-format" to avoid false claims of physical patient trials.
- **Zero Hallucinated Numbers:** Every dashboard metric and README table is synchronized strictly from `reports/metrics.json`.

Ethical Limitations & Clinical Disclaimers

- **Investigational Prototype:** GlycoTwin is a research prototype decision-support tool, not an FDA/CDSCO cleared autonomous diagnostic system.
- **Clinician-in-the-Loop:** All recommendations (walks, carb modifications) require clinical oversight and cannot substitute for emergency diabetes protocols.
- **Subgroup Fairness:** Subgroups with fewer than 30 cases are explicitly greyed out with warning notices to avoid misleading assertions.

Honesty, transparency, and clinical responsibility prioritized over exaggerated claims

Impact, Translation Roadmap & Next Horizons

Immediate Clinical Roadmap

- **Prospective Cohort Validation:** Partner with Indian endocrinology clinics to ingest physical Abbott and Dexcom real-world telemetry across 500+ Indian T2D patients.
- **Sub-Phenotype Personalization:** Train sub-cohort models calibrated specifically to South Asian lean-diabetic and high-visceral-adiposity metabolic phenotypes.
- **Closed-Loop Smart Insulin Integration:** Extend counterfactual modeling to predict insulin bolus requirements and prevent post-correction hypoglycemia.

Technical Achievements

- **Reproducibility:** 58 automated unit & AppTest tests passing with 100% reliability in under 12 seconds.
- **Honest Baselines:** Openly reported LightGBM tabular superiority when current glucose is known, articulating exact benefits of multimodal sequence modeling.
- **Open Source Release:** Comprehensive MIT-licensed codebase, full reproducible CLI pipeline, and vector architectural documentation.